

Living Well with IBS-D

A practical, non-medication guide to irritable bowel syndrome with diarrhoea.



IBS-D means recurring belly pain along with loose or frequent stools, often with urgency. It is real, it is common, and it settles best with several steady changes rather than one dramatic fix. Think of a gut that has its accelerator sensitised. The engine is fine, but the pedal responds to the lightest touch. Most of what follows is about calming that response.

Give any change 2 to 4 weeks before judging it, and change one thing at a time so you know what actually helped.

Get checked first if you notice any of these

- Blood in the stool, or black stools
- Weight loss you cannot explain, fever, repeated vomiting, or anaemia
- Diarrhoea that wakes you from sleep
- Severe or worsening pain
- Dizziness, fainting, or being unable to keep fluids down
- Oily, pale, hard-to-flush stools
- New bowel symptoms at an older age
- A family history of colon cancer, inflammatory bowel disease, or celiac disease
- Symptoms after recent antibiotics, a hospital stay, or a bout of travel diarrhoea
- Symptoms that persist despite sensible diet and lifestyle changes

Several other conditions can look like IBS-D, including infection, celiac disease, inflammatory bowel disease, thyroid overactivity and bile-acid problems. A doctor may order a few basic tests before calling it IBS. Do not label yourself.

1 Build a rhythm

- Eat at regular times. Long gaps followed by a big meal often set off urgency.
- Eat slowly and chew well.
- Keep sleep and wake times steady.
- Keep a symptom diary for 2 to 4 weeks: food and drink, stool form, urgency, any leakage, pain, bloating, stress, sleep.

The diary is there to spot patterns. It is not a reason to start cutting out half your diet.

2 Fluids and rehydration

Drink regularly, and more after loose stools. Around 2 litres a day suits many adults, adjusted for heat, activity and body size. If you have a heart, kidney or liver condition, check with your doctor first.

When diarrhoea is frequent or lasts more than a day or two, an oral rehydration solution works better than plain water because it replaces salt and sugar along with fluid. Use a ready-made sachet as directed.

3 Caffeine, alcohol and fizzy drinks

Caffeine speeds up the gut, and for many people that means urgency. Try keeping tea and coffee to about three cups a day, and cut back on energy drinks, alcohol and fizzy drinks. You do not have to quit everything. Watch your own response and adjust.

4 Move most days

Walking, cycling, swimming, yoga and strength work all help mood, sleep and gut symptoms. Start gently if you are inactive and build up. Time hard workouts away from your usual urgent hours.

5 Food

Start simple. Smaller meals, less very fatty or fried food, fewer fizzy drinks, and moderate caffeine and alcohol. Do not skip meals or fast to "rest the gut". It tends to backfire.

Sugar-free products deserve a look. Sorbitol and similar sweeteners in sugar-free sweets, chewing gum, "diet" drinks and some diabetic foods pull water into the gut and loosen stools. Check labels.

Fibre: soluble fibre, such as ispaghula (isabgol) husk, oats and barley, is the better choice. Coarse fibre like wheat bran can worsen bloating and urgency. Start with a small amount, raise it slowly, take it with water, and cut back if things get worse. More fibre is not automatically better.

The low-FODMAP diet helps some people with pain, bloating and stool problems. It is a short experiment with three phases, never a lifelong diet.

- **Restrict** for about 4 to 6 weeks.
- **Reintroduce** food groups one at a time and watch your response.
- **Personalise** to a varied diet with the foods you tolerate.

Do this with a dietitian who knows gut disorders. Skip it, or get supervision, if you have ever had an eating disorder, are losing weight, eat a very limited diet, or are pregnant.

Lactose and gluten: if symptoms reliably follow milk, a time-limited cut in lactose can make sense. Many people still handle yoghurt and hard cheese, so full dairy exclusion is rarely needed. A gluten-free diet is not routine for IBS. If celiac disease is possible, get tested before cutting gluten, since stopping it can hide the result. Some people who feel better off wheat are reacting to the wheat sugars, not gluten.

6 Living with urgency

- Keep to your eating pattern and avoid your known triggers before travel or big events.
- Know where the nearest toilets are when you are out.
- Carry a small kit: tissues, sanitiser, spare underwear.
- Do not restrict food or fluids to avoid going. It does not work and it drains you.
- Use the diary to tell food triggers apart from stress triggers.

If diarrhoea is getting in the way of work, travel or sleep, talk to your doctor. There are treatments for that, and you do not need to grit your teeth through it.

7 Calm the gut-brain loop

IBS is not imaginary. Gut and brain talk constantly through nerves, hormones and immune signals. Stress, poor sleep and fear of urgency turn up the volume on gut sensations, and the gut feeds that fear back. When the fear of an accident starts shrinking your life, that loop needs treatment too.

- 5 to 10 minutes of slow diaphragmatic breathing daily
 - Mindfulness or guided relaxation
 - Gentle yoga
 - Steady sleep and lower caffeine
 - Gradually returning to places and situations you have started avoiding
 - Gut-directed CBT or gut-directed hypnotherapy, which have some of the strongest evidence among psychological options for IBS
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8 Supplements and quick fixes

Probiotic evidence is mixed and depends on the strain, and no single product suits everyone. If you want to try one, use one product at a time, at the labelled dose, for up to about 12 weeks. Stop if nothing improves. A pricier multi-strain product is not automatically better.

Avoid colon cleanses, unregulated herbal mixes and "IBS cure" products. Quality and contamination are real concerns.

Medicines for diarrhoea, cramps and pain do exist, and some are useful for the right person. Please do not start antidiarrhoeal medicines, antibiotics or antidepressants on your own. Diarrhoea also should not be suppressed before a doctor has looked into it. The right choice depends on your main symptoms, other conditions, pregnancy and what else you take.

9 Your step-by-step plan

- 1 Set your baseline.** Rule out red flags, review recent antibiotics, travel and medicines, and start the diary. Pick one or two goals, such as fewer urgent trips or being able to leave the house comfortably.
 - 2 Use the basics for 2 to 4 weeks.** Regular meals, fluids, less caffeine, alcohol, fizzy drinks and sorbitol, gentle activity, and a cautious trial of soluble fibre.
 - 3 Review.** Are stools firmer, is urgency down, is pain better, and is your diet still varied?
 - 4 Escalate selectively.** Dietitian-led low-FODMAP trial, gut-directed therapy, further tests, or a medication discussion. See a gastroenterologist if symptoms are severe, unclear or not responding.
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Disclaimer: This guide is for general education only. It is not medical advice, diagnosis or treatment, and it does not create a doctor-patient relationship. Every person's gut is different. Please consult a qualified healthcare professional about your own symptoms before making changes to your diet, activity or treatment, and seek prompt care if you notice any warning signs listed above.